INFORMATION TO KEEP WITH YOU

My complete treatment handbook

Cisplatin + etoposide + chest radiotherapy

Understand your treatment. Know what to expect. Keep your plan to hand.

Displayed calendar
28 days
See the active treatment phase

Treatment stages
3
Explained in the treatment reference

Regimen source
NCCP 00456
Version 4

Cisplatin on Days 1 and 8; etoposide on Days 1–5. Each cycle lasts 28 days. Two cycles first; two more may be offered after review. Radiotherapy usually starts within 24 hours of the first chemotherapy dose; follow your separate radiation appointment plan.

Treatment stages

01 **Two initial cycles with radiotherapy**
Each cycle: 28 days

02 **Review before any further cycles**
Only if prescribed

03 **Recovery and next treatment decision**
Individual follow-up

Your team confirms the active stage and dates. The full treatment guide explains each step.

Inside your handbook

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My name _____

Hospital / oncology team _____

Use the urgent instructions and contact numbers in your treatment guide. Your team must replace example numbers before patient use. Your individual treatment plan takes precedence.

CHAPTER 01

Understanding your treatment

IN THIS CHAPTER

Why this treatment is offered, what its medicines do, and how the agreed plan fits your care.

Cisplatin + etoposide + chest radiotherapy
National Cancer Control Programme (NCCP) 00456

When to get help

Emergency: call 112 / 999

Severe breathing difficulty, collapse, seizure or severe chest pain: call emergency services. Do not wait for an online answer.

Contact your oncology service immediately

Temperature 37.5°C or higher, 36°C or lower, shivering or suddenly feeling unwell — even with a normal temperature. Call oncology immediately for fever, sudden illness, new or worsening breathlessness, troublesome bleeding, inability to drink or swallow, uncontrolled vomiting, very little urine or severe rash. Tell staff at once about pain, swelling, faintness or breathing trouble during an infusion.

Follow the urgent contact instructions from your treating hospital. If you cannot reach the team, seek urgent medical assessment. Outside clinic hours, use your instructed out-of-hours pathway or the emergency department.

This page does not diagnose symptoms or decide whether it is safe to stay at home. You do not need to know which medicine caused a symptom before calling.

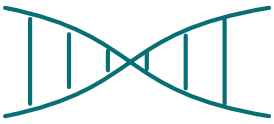
Chemotherapy and chest radiotherapy

Chemotherapy and chest radiotherapy work together to treat lung cancer. Your team will explain the aim, possible benefit and alternatives for you.

CHEMOTHERAPY

Cisplatin

Platinum chemotherapy



Damages cancer-cell deoxyribonucleic acid (the cell's genetic material) (DNA)

Cisplatin damages the DNA that cancer cells need to divide. It can also affect healthy cells.

DNA damage → Cell repair disrupted → Cell growth reduced

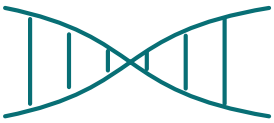
What to watch for

Report fever, unusual bleeding, reduced urine, hearing changes or tingling.

CHEMOTHERAPY

Etoposide

Topoisomerase II chemotherapy



Interferes with DNA repair

Etoposide interferes with an enzyme used to copy and repair DNA. This can stop cancer cells growing and can also lower healthy blood counts.

DNA copying → Repair interrupted → Cell growth reduced

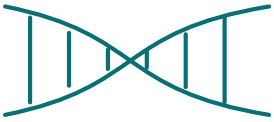
What to watch for

Report fever, bleeding, a sore mouth or feeling faint during an infusion.

RADIOTHERAPY

Chest radiotherapy

Local radiation treatment



Targets the planned chest area

Radiotherapy directs radiation at the planned chest area. Its appointments and dose are set by the radiation oncology team.

Planned chest area → Targeted radiation → Cancer-cell damage

What to watch for

Report painful swallowing, poor intake, skin changes, new cough or breathlessness.

The drawings are simplified explanations of drug action, not anatomical pictures or a prediction of response.

Understanding the evidence

Chemotherapy and radiotherapy are used together for selected people with Stage III lung cancer.

Published results help guide discussion but do not predict your outcome. The source study was small and did not compare this regimen with another treatment.

CHAPTER 02

Your schedule, tests and appointments

IN THIS CHAPTER

Your treatment stages, checks and practical preparations, from before treatment through follow-up.

How the treatments fit together

Treatment stages

01 **Two initial cycles with radiotherapy** Each cycle: 28 days	02 **Review before any further cycles** Only if prescribed	03 **Recovery and next treatment decision** Individual follow-up

Your team confirms the active stage and dates. The full treatment guide explains each step.

This shows treatment timing, not guaranteed symptom-free days. Your team confirms the active phase and dates.

EACH CYCLE: 28 DAYS

Two initial cycles with radiotherapy

1 2

Cisplatin on Days 1 and 8; etoposide on Days 1–5. Radiation appointments run alongside chemotherapy as planned by radiation oncology.

ONLY IF PRESCRIBED

Review before any further cycles

Optional 3 Optional 4

If the cancer has not progressed and treatment remains appropriate, the consultant may prescribe two additional cycles with an agreed radiation plan. These are not automatic.

INDIVIDUAL FOLLOW-UP

Recovery and next treatment decision

Review

Your team reviews recovery and response, then agrees the next plan. Further drug treatment or surgery is a separate decision.

The team confirms dates, radiation fractions and any optional phase.

BASELINE → TREATMENT → FOLLOW-UP

My tests and checks

Required safety tests are distinct from team-agreed scans and follow-up. Radiotherapy appointments and extra tests follow your personal plan.

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. NCCP 00456 v4 · official regimen · HPRA etoposide product information,14 November2025 · HSE lung cancer treatment: radiotherapy

THE WHOLE TREATMENT JOURNEY

Cisplatin + etoposide + chest radiotherapy: treatment and checks

Stage III non-small-cell lung cancer

Cisplatin on Days 1 and 8; etoposide on Days 1–5. Each cycle lasts 28 days. Two cycles first; two more may be offered after review. Radiotherapy usually starts within 24 hours of the first chemotherapy dose; follow your separate radiation appointment plan.

Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

Treatment, tests and reviews across the course

Care / check	Before treatment	Days 1–5	Day 8	Rest of cycle	After / review
Chemotherapy NCCP 00456 · ETOPOSIDE-SMPC	Confirm the plan	Both Day 1; etoposide Days 2–5	Cisplatin only	No planned chemotherapy	Review the next phase
Radiotherapy NCCP 00456 · ETOPOSIDE-SMPC	Planning and appointments	Starts close to chemotherapy	Follow radiation calendar	Continue if scheduled	Radiation follow-up plan
Blood / organ tests NCCP 00456 · ETOPOSIDE-SMPC	full blood count (FBC), kidneys / liver	FBC before etoposide doses	FBC and symptoms	Extra tests if needed	Recovery / individual scans

What each check is for

1 **Prepare both plans**

Before treatment

Chemotherapy and chest radiotherapy work together to treat lung cancer. Your team will explain the aim, possible benefit and alternatives for you.

Ask / act: Confirm chemotherapy, radiation, tests and contacts.

2

Review safety

Before each administration

Check bloods, organs and symptoms

Ask / act: Report fever, swallowing or breathing changes promptly.

3

Review next phase

After the planned cycles

Check response and recovery

Ask / **act:** Get the next plan in writing.

At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

Practical treatment planning

Agree the arrangements with your oncology team.

STEP 1

Before the first visit

Confirm both oncology teams, treatment and test appointments, medicines, transport and urgent contacts.

STEP 2

Frequent visits in week 1

Plan for chemotherapy on Days 1–5 plus Day 8. Radiation may require other visits; combine calendars with the team.

STEP 3

After the visits

Keep support medicines and contact numbers at hand. Ask early for help with swallowing, food, fluids or transport.

CHAPTER 03

Symptoms and supportive care

IN THIS CHAPTER

Recognising changes, knowing when to call, and using your prescribed supportive-care plan.

Day 1 → Day 28: a general pattern

This is a visual orientation, not a prediction of your personal cycle. It shows when some effects often emerge and which effects are **not** tied to a particular day.

Treatment or check	Timing in this view
Both chemotherapy medicines	 **Day 1** Day 1 after safety checks.
Etoposide only	 **Days 2–5** A separate infusion on each of Days 2,3,4 and 5, with prescribed safety checks.
Cisplatin only	 **Day 8** Day 8 after safety checks.
Radiotherapy: separate appointment plan	 **Days 1–28** This band shows overlap with the chemotherapy cycle, not daily radiation or a fixed number of fractions. Follow your radiation calendar.
Check symptoms throughout	 **Days 1–28** Symptoms can arise on any day, including after the last treatment.

Two overlapping plans: Chemotherapy days do not define the radiation schedule. Follow the written radiation calendar even on a day without chemotherapy.

Symptoms can build up: Tiredness and swallowing discomfort may develop during treatment. Breathing changes can also appear later; report them after treatment as well.

Why these bands stay broad: Relative chemotherapy days only. Radiotherapy band shows overlap, not a daily dose or fraction count. There is no guaranteed safe day for symptoms. Band length does not show likelihood or severity, and the chart cannot tell you whether it is safe to receive treatment. NCCP 00456 v4 · official regimen ↗

There are no guaranteed “safe days”. Call oncology immediately for fever, sudden illness, new or worsening breathlessness, troublesome bleeding, inability to drink or swallow, uncontrolled vomiting, very little urine or severe rash. Tell staff at once about pain, swelling, faintness or breathing trouble during an infusion.

Between-treatment notes

Day 1: Both chemotherapy medicines

Treatment after blood-test and symptom review. Radiotherapy usually begins close to this first treatment. Confirm both treatment plans and take only the prescribed support medicines.

Day 2–5: Etoposide each day

Return for etoposide on each of these four days. Radiotherapy appointments may also be planned. The team checks blood results and symptoms before dosing. Report swallowing or breathing changes.

Day 6–7: No planned chemotherapy

Follow the separate radiotherapy calendar. Anti-sickness medicines may continue as prescribed. Call if unwell; do not wait for the next infusion.

Day 8: Cisplatin only

Return for the second cisplatin dose after safety checks. Extra drip fluids are needed; etoposide is not given today. Confirm today's anti-sickness plan with the team.

Day 9–28: No further chemotherapy this cycle

Continue any planned radiotherapy appointments and safety checks. Tiredness and swallowing symptoms can build up. Report new symptoms and keep the urgent contact plan available, including after treatment ends.

Support around the medicines

1 **Your anti-sickness plan**

Cisplatin days need a different plan from etoposide-only days. The team combines prevention and rescue medicines into one clear schedule. Do not repeat or add courses yourself.

2 **Eating, drinking and swallowing**

Report painful swallowing, weight loss or difficulty drinking early. The team can arrange pain relief, dietitian support or other help.

3 **Fluids and kidney protection**

Follow your individual drinking advice and report much less urine. Cisplatin needs drip fluids before and after dosing; tell staff if you cannot drink or feel swollen or breathless.

4 **Other medicines and prevention**

Bring a full medicine list. Ask before starting medicines, supplements or vaccines. Fertility and contraception advice should be agreed before treatment.

5 Recovery before the next dose

Tell the team how severe symptoms were at their worst and what remains now, especially tingling, hearing change or difficulties with daily activities. Feeling better or reaching a planned date does not approve the next dose. The oncology team may delay, reduce or change a medicine while coordinating radiotherapy.

Questions worth asking

Do I attend every day?

Chemotherapy is planned on Days 1–5 and8 of each cycle. Radiotherapy has a separate appointment calendar; do not use this cycle diagram as the radiation timetable.

How many cycles will I have?

Two cycles are planned. Two further cycles may be offered only after review and a new confirmed plan.

Can side effects continue after treatment?

Yes. Keep reporting new or worsening symptoms after the last infusion or radiation session. The team agrees your follow-up checks.

Try explaining the plan back: Which chemotherapy days will I attend?,Where is my radiation calendar?,Who will I ring if unwell tonight?

Supportive prescription draft for review. This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

Day 1 · Cisplatin + etoposide

Active medicines: Cisplatin + Etoposide

Emetogenic risk: high

Medicine	Dose / route	When
Aprepitant	125 mg orally on the platinum day;80 mg on the next2 days	Relative to each cisplatin dose
Ondansetron	16 mg orally	Before the cisplatin dose Adjust for QT/individual risk.
Dexamethasone	12 mg orally on the platinum day;8 mg daily on the next3 days	Reference high-risk course Reconcile interactions and any other steroids.
Olanzapine	5 mg orally on the platinum day and next3 days	Reference high-risk course Sedation, metabolic risk and other medicines require review.

Medicine	Dose / route	When
Metoclopramide	10 mg orally three times daily as prescribed	Subsequent days NCIS high-risk row has no end-day orPRN qualifier. Confirm duration; coadministration with olanzapine increases extrapyramidal risk.

Additional phase-specific support

- Multiday systemic anticancer therapy (SACT): reconcile antiemetics for each day's medicines and the two days after; one combined prescription is required. Do not add independent overlapping courses.
- The5-day cisplatin aprepitant extension is not automatically applicable: this regimen gives cisplatin only on Days 1 and 8.
- Coordinate radiation-related nausea, swallowing, nutrition, skin care and respiratory assessment with radiation oncology.
- No automatic routine granulocyte colony-stimulating factor (white-cell growth support) (G-CSF), antibacterial, antiviral or Pneumocystis jirovecii pneumonia (PJP) prophylaxis is specified in these NCCP regimens; consider individual risk and local policy.
- Review etoposide interactions, live vaccines, nephrotoxic/ototoxic medicines and any duplicate QT-prolonging antiemetics.
- Cisplatin pre/post hydration and electrolytes per local policy. NCCP suggests posthydration10 mmol magnesium sulphate plus20 mmol potassium chloride in 1 L sodium chloride0.9% over2 hours; adjust to patient and local policy.
- Day 1 etoposide in 1 L may be considered prehydration; Day 8 has no etoposide infusion, so prescribe its own prehydration. Routine furosemide to induce urine flow is not recommended without fluid overload.
- NCCP reduced count rows still require reconciliation with the etoposide SmPC. Count rows apply to etoposide and do not clear cisplatin independently.
- After grade 3 non-haematological toxicity (excluding mucositis/alopecia), wait for recovery to grade 1 or less; the source 75% restart requires dose-history and denominator review. Grade 4 is outside that source row.
- After grade ≥2 neuropathy recovers to ≤1, a consultant chooses carboplatin AUC5 substitution or 50% cisplatin. Do not restore full-dose cisplatin automatically or substitute the separate AUC3 regimen.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

Days 2–5 · etoposide only

Active medicines: Etoposide

Emetogenic risk: low

Medicine	Dose / route	When
Ondansetron	8 mg orally once daily	Before the etoposide-only dose Reconcile any existing delayed-emesis5-HT3 plan; do not duplicate.
Metoclopramide	10 mg orally up to three times daily as required	Rescue use as prescribed Prescriber confirms suitability and duration.

Additional phase-specific support

- Multiday systemic anticancer therapy (SACT): reconcile antiemetics for each day's medicines and the two days after; one combined prescription is required. Do not add independent overlapping courses.
- The5-day cisplatin aprepitant extension is not automatically applicable: this regimen gives cisplatin only on Days 1 and 8.
- Coordinate radiation-related nausea, swallowing, nutrition, skin care and respiratory assessment with radiation oncology.
- No automatic routine granulocyte colony-stimulating factor (white-cell growth support) (G-CSF), antibacterial, antiviral or Pneumocystis jirovecii pneumonia (PJP) prophylaxis is specified in these NCCP regimens; consider individual risk and local policy.
- Review etoposide interactions, live vaccines, nephrotoxic/ototoxic medicines and any duplicate QT-prolonging antiemetics.
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Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

Day 8 · Cisplatin only

Active medicines: Cisplatin

Emetogenic risk: high

Medicine	Dose / route	When
Aprepitant	125 mg orally on the platinum day;80 mg on the next2 days	Relative to each cisplatin dose
Ondansetron	16 mg orally	Before the cisplatin dose Adjust for QT/individual risk.
Dexamethasone	12 mg orally on the platinum day;8 mg daily on the next3 days	Reference high-risk course Reconcile interactions and any other steroids.
Olanzapine	5 mg orally on the platinum day and next3 days	Reference high-risk course Sedation, metabolic risk and other medicines require review.

Medicine	Dose / route	When
Metoclopramide	10 mg orally three times daily as prescribed	Subsequent days NCIS high-risk row has no end-day orPRN qualifier. Confirm duration; coadministration with olanzapine increases extrapyramidal risk.

Additional phase-specific support

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- No automatic routine granulocyte colony-stimulating factor (white-cell growth support) (G-CSF), antibacterial, antiviral or Pneumocystis jirovecii pneumonia (PJP) prophylaxis is specified in these NCCP regimens; consider individual risk and local policy.
- Review etoposide interactions, live vaccines, nephrotoxic/ototoxic medicines and any duplicate QT-prolonging antiemetics.
- Cisplatin pre/post hydration and electrolytes per local policy. NCCP suggests posthydration10 mmol magnesium sulphate plus20 mmol potassium chloride in 1 L sodium chloride0.9% over2 hours; adjust to patient and local policy.
- Day 1 etoposide in 1 L may be considered prehydration; Day 8 has no etoposide infusion, so prescribe its own prehydration. Routine furosemide to induce urine flow is not recommended without fluid overload.
- NCCP reduced count rows still require reconciliation with the etoposide SmPC. Count rows apply to etoposide and do not clear cisplatin independently.
- After grade 3 non-haematological toxicity (excluding mucositis/alopecia), wait for recovery to grade 1 or less; the source 75% restart requires dose-history and denominator review. Grade 4 is outside that source row.
- After grade ≥2 neuropathy recovers to ≤1, a consultant chooses carboplatin AUC5 substitution or 50% cisplatin. Do not restore full-dose cisplatin automatically or substitute the separate AUC3 regimen.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

What to watch for

You do not need to know which medicine caused a symptom before calling.

1 Hearing / nerves

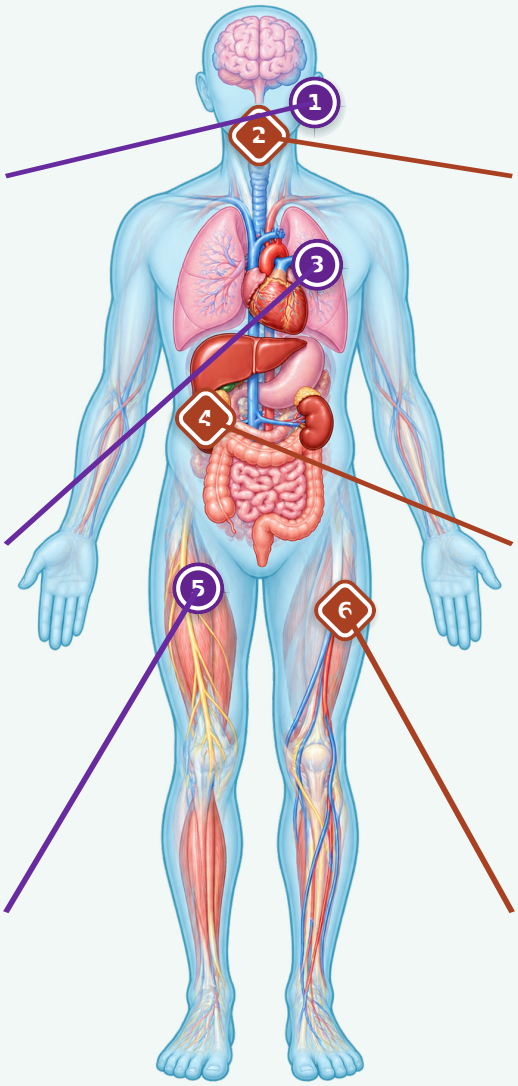
Cisplatin

Report ringing in the ears, hearing change or new tingling.

2 Mouth / swallowing

Chemotherapy + chest radiotherapy

Report mouth sores, painful swallowing or difficulty drinking.



3 Breathing

Chest radiotherapy + chemotherapy

New or worsening cough or breathlessness needs prompt advice, during or after treatment.

4 Kidneys / fluids

Cisplatin

Report much less urine, swelling or trouble drinking.

5 Blood cells

Chemotherapy

Fever, chills, sudden illness or unusual bleeding need immediate advice.

6 Skin / energy

Chest radiotherapy + chemotherapy

Tell the team about sore skin in the treated area, a spreading rash or increasing fatigue.

Selected effects, not every possible effect or a prediction. Blood-count, skin and line markers are schematic. Number and text carry meaning; colour is decorative.

Call oncology immediately for fever, sudden illness, new or worsening breathlessness, troublesome bleeding, inability to drink or swallow, uncontrolled vomiting, very little urine or severe rash. Tell staff at once about pain, swelling, faintness or breathing trouble during an infusion.

When to get help

Call 112 / 999 for severe breathing difficulty, severe chest pain, collapse or a seizure.

Call oncology immediately: temperature 37.5°C or above, 36°C or below, shivering or suddenly feeling unwell, even with a normal temperature. Follow your hospital's temperature instructions. If you cannot reach the team, seek urgent medical assessment. Do not wait for the next appointment.

Oncology daytime: 087 12345678 · **Out of hours:** 087 87654321

EXAMPLE ONLY — team must replace both numbers before patient use.

CHAPTER 04

Your record and contact card

IN THIS CHAPTER

Space for your own cycle or review record, questions and notes, followed by your treatment card.

Record each cycle separately. Each medicine has its own symptom table. Mark the worst symptoms since your last review, when they occurred and what they stopped you doing. Overlapping symptoms can be recorded under more than one medicine; you do not have to identify the cause.

These are patient-language severity prompts, not a validated questionnaire or a formal Common Terminology Criteria for Adverse Events (CTCAE) grading tool. Your team assigns the clinical grade. Serious symptoms override the columns: call without waiting to decide a severity level.

Use a new record sheet for each cycle or review. Copy or print another sheet when needed; the number of sheets does not prescribe the length of treatment.

Cycle number: _____ Start date: _____ Next review: _____

Record actual treatment dates, delays and omitted doses. A completed cycle includes all planned chemotherapy doses; it does not approve the next dose or record each radiation fraction.

28-day chemotherapy cycle · copy this sheet for another cycle of the same phase.

Cisplatin · Chemotherapy

Symptom	None	Mild	Moderate / call	Severe / urgent	Emergency signs
Nausea	□ None	□ Queasy but eating and drinking	□ Intake reduced: contact the team	□ Unable to maintain intake: urgent review	□ Collapse / severe dehydration: 112/999
Fatigue	□ Usual energy	□ Tiredness improves with rest	□ Rest does not help; usual tasks affected: call	□ Self-care difficult: urgent review	□ Collapse: 112/999
Numbness / tingling	□ None	□ Mild change; usual tasks unaffected	□ Limits buttons, writing or walking: call	□ Washing, dressing or walking difficult: urgent review	□ Sudden major weakness / paralysis: emergency help
Urinary symptoms	□ Usual pattern	□ Mild change: report	□ Pain or frequent urination: call	□ Cannot pass urine / marked bleeding: urgent review	□ Collapse or severe illness: 112/999

Etoposide · Chemotherapy

Symptom	None	Mild	Moderate / call	Severe / urgent	Emergency signs
Nausea	□ None	□ Queasy but eating and drinking	□ Intake reduced: contact the team	□ Unable to maintain intake: urgent review	□ Collapse / severe dehydration: 112/999
Mouth soreness / ulcers	□ None	□ Mild soreness; eating normally	□ Painful; soft food needed: call	□ Cannot eat / drink enough: urgent review	□ Major breathing difficulty: 112/999
Fatigue	□ Usual energy	□ Tiredness improves with rest	□ Rest does not help; usual tasks affected: call	□ Self-care difficult: urgent review	□ Collapse: 112/999
Bleeding / bruising	□ None	□ New small bruise / brief bleed: report	□ Repeated bleeding: call today	□ Bleeding that will not stop: urgent help	□ Heavy bleeding / collapse: 112/999

Dates / doses missed or changed / symptoms affecting daily life / questions:

Red flags override the table. Fever, shivers, suddenly unwell, chest symptoms or inability to drink: seek prompt advice. Severe breathing difficulty, collapse or severe chest pain: 112 / 999.

Cycle number: _____ Start date: _____ Next review: _____

Record actual treatment dates, delays and omitted doses. A completed cycle includes all planned chemotherapy doses; it does not approve the next dose or record each radiation fraction.

Separate radiotherapy appointment record · copy this sheet for another cycle of the same phase.

Chest radiotherapy · Radiotherapy

Symptom	None	Mild	Moderate / call	Severe / urgent	Emergency signs
Fatigue	□ Usual energy	□ Tiredness improves with rest	□ Rest does not help; usual tasks affected: call	□ Self-care difficult: urgent review	□ Collapse: 112/999
Mouth soreness / ulcers	□ None	□ Mild soreness; eating normally	□ Painful; soft food needed: call	□ Cannot eat / drink enough: urgent review	□ Major breathing difficulty: 112/999
Breathing / cough	□ Usual breathing	□ New cough or breathlessness: call promptly	□ Symptoms with usual activity: urgent review	□ Breathless at rest: urgent assessment	□ Severe breathing difficulty / collapse: 112/999
Skin / nail changes	□ None	□ Mild change: report	□ Pain or daily tasks affected: call	□ Spreading infection or blistering: urgent review	□ Severe illness / collapse: emergency help

Dates / doses missed or changed / symptoms affecting daily life / questions:

> Red flags override the table. Fever, shivers, suddenly unwell, chest symptoms or inability to drink: seek prompt advice. Severe breathing difficulty, collapse or severe chest pain: 112 / 999.

Keep your treatment details to hand

Stage III non-small-cell lung cancer · NCCP 00456 v4 · Document revised 7 November 2026

Print this page **one-sided on A4 at Actual size / 100%**. Cut only the dashed outer border around both panels. **Do not cut the centre line.** Fold with FRONT and BACK facing out, then glue or tape the blank backs together. Finished size: **85.6 × 54 mm**. Keep the stamp side writable.

FRONT / CONTACTS

My Treatment Card

SACTCheck

Name: _____

Cisplatin + etoposide + chest RT

28-day cycles: Cisplatin D1/8; etoposide D1–5; chest RT. 2 cycles, then review.

Phase: _____ Cycle plan: team to confirm

Daytime (EXAMPLE) 087 12345678

Out of hours (EXAMPLE) 087 87654321

Unwell or fever? Call oncology now.
Severe breathing difficulty, chest pain or collapse: **112 / 999.**

EXAMPLE NUMBERS — replace before patient use.

BACK / TREATMENT RECORD

My cycle record

Rolling record — follow your agreed treatment plan.

Cycle	Date / initials

My treatment information

Latest treatment date: _____

Plan changed? Ask the team to update this card.

CUT outer border

FOLD centre — do not cut

Keep the record accurate

The team dates and initials each recorded treatment unit when complete. A cycle is complete after its final planned dose; a weekly-treatment stamp records that weekly dose. Delays, omitted doses or a changed plan need a note or a replacement card. A stamp does not approve the next treatment. Keep any medicine-specific alert card too.

Put your treatment page on your phone

Scan your QR and check the treatment name. Save that page to your home screen.

iPhone / Safari

Tap Share → Add to Home Screen. Select Open as Web App if offered, then Add.

Android / Chrome

Tap the three-dot menu → Add to Home screen or Install and create shortcut. Choose the shortcut option and confirm.

Menu wording varies. The QR opens information, not a personal record. Paper stamps do not update the website. Check that the saved icon opens the correct treatment.

Your local contacts and resources

Oncology daytime: 087 12345678 · **Out of hours:** 087 87654321

EXAMPLE ONLY — team must replace both numbers before patient use.

Hospital / team: _____
Backup if the team cannot be reached: _____

Additional local services and resources will be added when confirmed. Your team must check and complete this page before patient use.

SACTCheck v0.84.45 · NCCP 00456 v4 · Wallet-card sheet
Feasibility build. Clinical, pharmacy and patient/PPI review pending. Your team confirms your personal plan.

CHAPTER 05

Sources and further information

IN THIS CHAPTER

The references behind this information and addresses you can use to find further help.

Use this information alongside the plan agreed with your treatment team. Your team confirms your medicines, doses and appointments.

1. **NCCP 00456 v4 · pNCCP 00456 supportive care; NCIS V8 Table 1 and multiday section**
   https://healthservice.hse.ie/documents/6902/456_V4_CISplatin_50_and_Etoposide.pdf
2. **HPRA etoposide product information,14 November2025**
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3. **HSE lung cancer treatment: radiotherapy**
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